Methotrexate: head and neck cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1, 8 and 15: methotrexate

Active medicines: Methotrexate

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg PO OD	Each prescribed methotrexate treatment day (Days 1, 8 and 15) Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	PRN Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.
- Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.
- Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.
- Check interactions before any local adjunct: methotrexate clearance and toxicity can be affected by other medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-14 · SACTCheck v0.84.50 · Regimen-level drafts; prescriber and oncology-pharmacy verification required